

OPERATIONS & COMPLIANCE

Scope of Practice, Credentialing & Training

Verifying who may do what, proving they were trained, and keeping it current

DOCUMENT OPS-003	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Practice manager, medical director, all clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol answers the question that follows every adverse event: who authorized this person to perform this procedure, and what evidence is there that they were competent to do it. It covers license verification, the service-by-service competency matrix, onboarding, and ongoing review.

TWO SEPARATE QUESTIONS

Scope is what a license permits in your state. **Competency** is whether this particular person can safely do this particular thing. A procedure can be within scope and still be unsafe to delegate. Both need to be documented, and neither substitutes for the other.

2 Credential Verification

When	What is verified	Record
At hire	Primary source verification of license or certification directly with the issuing board — not a photocopy supplied by the candidate. Confirm status, expiry, and any disciplinary history.	Screenshot or printout with the date checked and the name of the verifier.
At each renewal	Re-verification at primary source. Diarize renewal dates in advance rather than discovering a lapse.	Updated verification record; prior records retained.
At hire and annually	BLS certification currency for all clinical staff, and ACLS where the medical director requires it.	Copy of the card with expiry tracked.
At hire	Professional liability coverage where the individual carries their own, plus confirmation of coverage under the practice policy.	Certificate of insurance on file.
Ongoing	Continuing education relevant to the services performed, at the interval required by licensure and by the medical director.	CE log with certificates retained.

A LAPSED LICENSE IS NOT A PAPERWORK PROBLEM

Treatment performed by someone whose license lapsed is generally uninsured, may constitute unlicensed practice, and exposes both the individual and the practice. Renewal tracking is a named person's explicit responsibility, not a shared assumption.

3 Competency Matrix

Build one row per service offered and one column per staff member. Each cell is authorized or not, dated and signed by the medical director. This takes about an hour to populate and answers the hardest question you will be asked.

REQUIRED FIELDS PER AUTHORIZATION

Field	Detail
Service	The specific procedure, at the level of granularity used in your clinical SOPs.
Minimum credential	License level required in your state, plus any device or manufacturer certification.
Training completed	What training, delivered by whom, on what date.
Supervised cases	Number of supervised procedures completed before independent practice, and by whom they were supervised.
Assessment	How competency was assessed — direct observation is the standard.
Authorized by	Medical director signature and date.
Review date	When this authorization is next reviewed.

4 Onboarding a New Clinical Hire

- 1 Verify credentials at primary source** before the first patient contact. No exceptions for experienced hires or short-notice cover.
- 2 Orient to emergency infrastructure.** Kit, AED and oxygen located and the emergency code learned on day one, before any clinical work.
- 3 Issue the protocol set** for the services they will perform. Record that they were issued and read.
- 4 Observation period.** The new hire observes the practice's technique for each service before performing it.
- 5 Supervised practice.** A defined number of cases per service, directly supervised, with each case recorded.
- 6 Competency assessment** by direct observation, against the clinical SOP rather than against habit.
- 7 Medical director authorization** signed into the competency matrix, service by service.
- 8 Enhanced chart review** for a defined initial period, per OPS-001.
- 9 Probation review** at a set date, with any restrictions recorded.

5 Ongoing Review & Scope Boundaries

- ☐ Annual competency review per staff member, per service, signed by the medical director.
- ☐ Re-assessment required after any extended absence from a service.

- ☐ Re-assessment required when a device, product or technique changes materially.
- ☐ Adverse event patterns trigger targeted review rather than an informal conversation.
- ☐ CE requirements tracked with a named owner and advance reminders.
- ☐ Matrix updated within a week of any staffing change.

When a task falls outside authorization

- Staff are explicitly authorized — and expected — to decline any task they are not signed off for, without needing to justify it in the moment.
- Escalate to the supervising provider or medical director rather than proceeding.
- Commercial pressure, a full schedule and an unhappy patient are never grounds to proceed outside authorization. Record the escalation and the outcome.
- Repeated pressure to work outside scope is itself an incident and is reported to the medical director.

MAKE DECLINING SAFE

A competency matrix only works if staff feel able to use it. State plainly, in writing and in onboarding, that declining an unauthorized task is the expected behaviour and will never be held against anyone. Practices where that is not culturally true get the risk anyway, just undocumented.

6 Quick Reference

QUICK REFERENCE CARD

Before anyone treats a patient

1. **License verified at primary source** — not a photocopy.
2. **BLS current** and on file.
3. **Emergency kit, AED and oxygen located** on day one.
4. **Protocols issued and read**, recorded.
5. **Observed, then supervised** for a defined number of cases.
6. **Competency assessed by direct observation.**
7. **Medical director signed the matrix** for that specific service.
8. **Not signed off? Decline and escalate.** Always the right answer.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

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